NCCP 00455 · clinician draft

Pembrolizumab 200 mg · colorectal

Colorectal treatment context This guide covers first-line metastatic MSI-H/dMMR colorectal cancer. The shared NCCP regimen has other cancer indications, but their tests, treatment aims and durations are not this guide.

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Pembrolizumab 200 mg every 3 weeks

Active medicines: pembrolizumab

Emetogenic risk: Minimal

No routine prophylactic antiemetic order is populated for this phase. Agree an individual symptom-treatment plan where needed.

Additional phase-specific support

- No routine antiemetic prophylaxis or routine infusion premedication is usually required. Use an individual/local symptom-relief prescription if needed.
- Carry the immunotherapy alert card and report possible immune symptoms promptly during and after treatment.
- Effective contraception during treatment and at least 4 months after the last dose.
- For this colorectal context, eligibility is first-line MSI-H/dMMR metastatic disease; PD-L1 is not the colorectal selection test. Do not import a lung adjuvant course cap.
- FBC, renal/liver profile and glucose before each cycle. Thyroid tests every 3–6 weeks: keep thyroid checks when due regardless of the infusion interval. Cortisol as clinically indicated.
- Review any immune toxicity and steroid dose before restarting; no routine dose reduction. Delayed immune effects may occur after treatment stops.
- If treatment is paused because of an immune side effect, your team checks recovery and any steroid or hormone plan before deciding whether it can restart. Do not change steroids or restart treatment yourself.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

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Sources checked 2026-11-05 · SACTCheck v0.84.35 · Regimen-level drafts; prescriber and oncology-pharmacy verification required